

PHYSICAL THERAPY DISCHARGE SUMMARY

Patient	Mary Lou Bevredeau	Treatment dates	10/01/2025 to 01/08/2026
DOB / Sex	06/21/1961 / Female	Referring physician	Anita Kessler, MD, Northshore Spine Institute
Insurance	Lakeshore Mutual Health	Treating therapist	Jordan Ellis, PT, DPT
Member ID	LMH-448219037	Diagnosis	Lumbar radiculopathy, left lower extremity (ICD-10 M54.16); low back pain (ICD-10 M54.50)

INTAKE HISTORY - INITIAL EVALUATION 10/01/2025

History of present illness: 64-year-old female referred for evaluation and treatment of low back pain with left leg radiating pain that began in June 2025 without a specific injury. Pain radiates from the left buttock into the lateral thigh and lower leg. Symptoms increase with prolonged standing and walking (tolerance approximately 20 minutes) and with sitting longer than 30 minutes. Patient reports difficulty with household chores and yard work. No prior physical therapy for this episode. Denies bowel or bladder changes.

Past medical history: Hypertension (controlled), hyperlipidemia.

Social history: current smoker, 1/2 pack per day. Occasional alcohol use. Lives with spouse in a single-story home. Retired school administrator.

Patient goals: Walk 30 minutes without increased leg pain; return to gardening; sleep through the night without waking from pain.

INITIAL OBJECTIVE FINDINGS

Measure	Initial Evaluation (10/01/2025)
Pain (NPRS, current / worst)	7/10 / 9/10
Oswestry Disability Index	46% (severe disability)
Lumbar flexion	Limited approximately 50%, reproduces left leg symptoms
Lumbar extension	Limited approximately 25%, increases low back pain
Straight leg raise	Positive on the left at 45 degrees; negative on the right

COURSE OF TREATMENT

The patient attended **24 visits over 14 weeks** (10/01/2025 to 01/08/2026), with treatment frequency of approximately two sessions per week. Attendance was consistent; there were no unexcused missed visits. Treatment followed a lumbar stabilization and neural mobility plan of care.

Visit Summary

First visit	Last visit	Total visits	Duration	Frequency	Unexcused missed
10/01/2025	01/08/2026	24	14 weeks	~2x / week	0

Interventions

Intervention	Description
Core stabilization	Progressive transversus abdominis and multifidus activation, dead bug and bird dog progressions, bridging, and loaded carries.
Manual therapy	Lumbar and hip joint mobilization (grades I to III), soft tissue mobilization to lumbar paraspinals and left gluteals, and sciatic nerve gliding.
Home exercise program	Daily program of stabilization, nerve glides, and walking, issued at visit 2 and progressed at each progress note. Patient reported good adherence.
Supporting interventions	Postural education, body mechanics training for lifting and household tasks, and moist heat as an adjunct.

Progress Notes Summary

Date	Visit #	Pain	Summary
10/29/2025	8	6/10	Improved sitting tolerance to about 45 minutes. Walking tolerance unchanged. Leg symptoms persist with standing.
12/03/2025	15	5/10	5/10 on good days; intermittent flares to 8/10 after activity. Partial improvement in lumbar flexion. Leg symptoms unchanged in distribution.
01/08/2026	24	6/10	Gains from early weeks not sustained. Function has plateaued. Discharge from skilled therapy.

Summary of response: The patient showed limited, temporary improvement during the course of care, mainly in sitting tolerance and trunk mobility. Improvements were not sustained, and left leg radiating pain continued to limit standing and walking. The patient remained adherent to the plan of care and home exercise program throughout.

DISCHARGE EXAMINATION - 01/08/2026

Measure	Discharge Findings
Lumbar flexion	Limited approximately 40%, reproduces left leg symptoms
Lumbar extension	Limited approximately 25%
Straight leg raise	Positive on the left at 50 degrees; negative on the right
Reflexes	Patellar and Achilles 2+ and symmetric
Gait	Independent without assistive device; reduced stride length after 15 minutes of walking

Strength Exam (Manual Muscle Testing)

Muscle group	Right	Left
Hip flexion	5/5	5/5
Hip abduction	5/5	5/5
Knee extension	5/5	5/5
Ankle dorsiflexion	5/5	5/5
Ankle plantarflexion	5/5	5/5

Summary: strength 5/5 in both lower extremities.

Pain and Function Scores

Measure	Intake (10/01/2025)	Discharge (01/08/2026)
Pain, current (NPRS)	7/10	6/10
Pain, worst (NPRS)	9/10	8/10
Oswestry Disability Index	46%	42%

Goal Status

Goal	Status
Walk 30 minutes without increased leg pain	Not met (current tolerance about 15 to 20 minutes)
Return to gardening	Partially met (light tasks only, with rest breaks)
Sleep through the night without waking from pain	Not met

DISCHARGE STATUS

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| ☐ Discharged - goals met | ☒ Discharged - progress plateaued |
| ☐ Discharged - patient self-discharge | ☐ Discharged - non-attendance |
| ☒ Home exercise program issued / continue | ☒ Referred to physician: surgical consultation |

Assessment and outcome: Patient completed a 14-week course of physical therapy with good adherence. Response has **plateaued**, with only limited and temporary improvement in pain and function, and persistent left leg radiating pain. Further skilled physical therapy is not expected to produce meaningful additional gains at this time.

Recommendation: Discharge from physical therapy. **Recommend surgical consultation** with Dr. Kessler for further evaluation. Patient to continue the home exercise program.

Electronically signed: Jordan Ellis, PT, DPT, Prairie Path Physical Therapy | 01/08/2026